
WHO Country Cooperation Strategy

Spain
2024–2030



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Abbreviations

AECID	Spanish Agency for International Development Cooperation
AMR	antimicrobial resistance
ATACH	Alliance for Transformative Action on Climate and Health
CCS	Country Cooperation Strategy
COVID-19	coronavirus disease
ECEH	European Centre for Environment and Health
EHP	Environment and Health Process
EMT	emergency medical teams
EPW	The European Programme of Work, 2020–2025 – “United Action for Better Health”
GPW14	Fourteenth General Programme of Work
NCDs	noncommunicable diseases
NTDs	neglected tropical diseases
PAHO	Pan American Health Organization
PHC	primary health care
SDG	Sustainable Development Goal
SNS	Spanish National Health System
STIs	sexually transmitted infections
TB	tuberculosis
UHC	universal health coverage
UNAIDS	Joint United Nations Programme on HIV and AIDS

Foreword



Mónica García Gómez
Minister of Health, Spain

Spain and the World Health Organization are delighted to present their Country Cooperation Strategy 2024–2030. This ambitious roadmap for collaboration embodies a shared commitment to healthy societies, a thriving planet, and robust and equitable national and global health systems. It is fully coherent with Spain’s national and international health and development agenda as well as WHO’s global and regional strategic planning frameworks.



Hans Henri P. Kluge
WHO Regional Director for Europe

Going forward, Spain and WHO’s joint work will be shaped by five mutually agreed strategic priorities. These reflect an explicit acknowledgement that health is a collective endeavour, one that requires not only individuals, but also communities, countries and continents to work together. Spain’s vision for safeguarding the health of its own population is thus indivisible from its contribution to WHO’s wider mission and to all the Sustainable Development Goals.

Recognizing its responsibility to exercise global health leadership commensurate with the complexity of global health challenges, Spain will work to bolster the quality and sustainability of health systems, reinforcing the foundations of universal health coverage with primary health care as a fundamental pillar; strengthen accountability to tackle the impacts of the climate crisis on health; reinforce the multilateral framework for health emergency prevention, preparedness and response; and promote equity, gender equality and human rights worldwide. For its part, all levels of WHO will be deployed over the coming years to amplify Spain’s expertise and influence abroad to the benefit of other Member States it serves, and to support Spain’s public health objectives at home.

Executive summary

The present Country Cooperation Strategy (CCS) between Spain and WHO is a non-legally binding document, which sets out a medium-term strategic agenda for collaboration from 2024 to 2030. Taking into account the sociodemographic, epidemiological and health system characteristics of Spain along with the current areas of domestic, regional and global collaboration with WHO, the CCS charts out five strategic priorities for the coming years, each supported by a subset of deliverables and key interventions.

Spain is a high-income country, home to just under 50 million people. The population is urbanized and diverse, with markedly differentiated regions and nearly a fifth of residents born outside Spain. Governance is highly decentralized, with the central government taking on a regulatory and strategic – rather than operational or management – role. Indicators for both population health and health system performance are very favourable, thanks in large part to a high-quality, universally accessible health system based on a primary health care model. At the same time, public health also faces major challenges, including an ageing population and health-care workforce; the intensifying effects of the climate crisis; the accelerating cost-of-living crisis, especially in relation to housing; the ever-present threat of health emergencies; and persistent inequalities rooted in the social determinants of health.

The Spanish Ministry of Health’s vision for the future hinges on restoring the country’s collective pride and trust in public health care; adapting and expanding services to better meet population needs; and strengthening governance and intersectoral approaches. There is also broad consensus on the interdependence between domestic and global health, and the country has taken decisive steps to fulfil its responsibilities to the international community through multilateral approaches, rooted in the achievement of the Sustainable Development Goals. Spain is an active contributor to WHO’s work at “home” in the European Region, but it also plays a key role in WHO globally and in other WHO regions. In particular, Spain has developed an excellent relationship with its partners in the Americas. It exercises significant leadership in technical areas where Spain’s performance excels, including primary health care, gender equality and organ transplantation, among others.

Through the CCS, Spain aims to amplify the international impact of its health expertise and leadership, strengthen global systems to respond to borderless health threats, and exchange experiences and best practices with peer countries coping with similar domestic challenges. Five strategic priorities for its collaboration with WHO have been designated, in alignment with the national Public Health Strategy, WHO’s Fourteenth General Programme of Work and European Programme of Work, 2020–2025 – “United Action for Better Health”, and the Pan American Health Organization’s Strategic Plans for the 2020–2025 and 2026–2031 periods:

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1. Reinforce the foundations of **universal health coverage** through health workforce planning and team care, transformation of primary health care, and increased access to complex services.
 2. Address **noncommunicable diseases** through an evidence-based, intersectoral, community- and person-centred approach.
 3. Strengthen **multilateral approaches to health emergencies** to reduce risk and ensure all countries are better prepared and equipped to respond.
 4. Accelerate a just transition towards a **climate-resilient, sustainable health system and society**.
 5. Strengthen the control of **infectious diseases** at a global level, including neglected tropical diseases and sexually transmitted infections as well as antimicrobial resistance.

These priority areas of work are supported by a transversal commitment to four other cross-cutting principles that will underpin all actions: health and human rights, social and health equity, gender equality, and One Health.

In turn, each strategic priority is accompanied by a set of deliverables and key interventions, which will be implemented by a wide array of institutional actors in both Spain and at all levels of WHO. In order to maximize the agility of implementation, a parallel work plan has been developed as a “live” instrument, accessible to both WHO and Spain’s Ministry of Health. It will be updated on an ad hoc basis to reflect adjustments in institutional arrangements, resource availability and objectives. To ensure coordination and coherence, on the one hand, while avoiding heavy reporting or monitoring and evaluation exercises, on the other, a single focal point from the Spanish Ministry of Health and the WHO Regional Office for Europe will be designated to track implementation on each side. Together, they will organize yearly check-ins to take stock of progress, identify bottlenecks and adapt implementation mechanisms as needed.



Introduction and scope

This Country Cooperation Strategy (CCS) sets out a medium-term strategic agenda for the collaboration between Spain and WHO in the 2024–2030 period. The CCS is a legally non-binding policy instrument that aims to articulate and explain how WHO will work with Spain to promote public health domestically and internationally, ensuring coherence in the cooperation between Spain and WHO overall and in jointly identified priority areas.

The CCS defines, on the one hand, how Spain will support WHO in implementing their Fourteenth General Programme of Work (GPW14) (1), the European Programme of Work, 2020–2025 – “United Action for Better Health” (EPW) (2), and the Pan American Health Organization (PAHO) strategic plans (3,4) and how the parties will work together to meet other international health and development commitments, for example the UHC2030 pledge (5) to contribute to stronger health systems worldwide. On the other hand, the CCS outlines how WHO will harness the resources and know-how of the international public health community to support Spain in implementing its national health objectives and achieve the Sustainable Development Goals (SDGs) in its own territory.

Situation analysis

Country context and population health at a glance

Spain is a high-income country and Organization for Economic Cooperation and Development member, home to approximately 48.6 million people (6). Most of the population (84.1%) lives in urban areas, which occupy 15% of the territory, while nearly one fifth of inhabitants (18.9%) were born outside Spain, and a similar proportion (20.4%) is over 65 years old. Life expectancy is among the highest in the world, standing at 83.2 years in 2019 (7), which the country has managed to achieve with a relatively modest level of health expenditure (around €1927 per capita in 2022 (8)). Factors contributing to good population health and well-being include universal healthcare coverage (UHC) based on a primary health care (PHC) model; strong social ties and social capital; a Mediterranean diet; a temperate climate favourable to year-round outdoor physical activity; and adequate social protection schemes, including – since 2020 – a guaranteed minimum basic income.

However, these strengths coexist with major challenges to public health, also common to peer countries in Europe. Rapid population (and workforce) ageing is impacting the disease burden and health system capacity, while climate change and environmental degradation are bringing extreme weather events, shortages of freshwater, and a growing risk of zoonotic diseases. In addition, the accelerating cost-of-living crisis is outpacing reforms to labour standards and the expansion of welfare programmes. In 2023, about 26.5% of the population was at risk of poverty or social exclusion. This figure is highest among children and varies by region (9).

Health system highlights

The right to health protection is enshrined in Article 43 of the Spanish Constitution (10). The Spanish health-care model is based on the principle that everyone has the right to health, regardless of their economic, employment or territorial situation. The Spanish National Health System (SNS) provides comprehensive, universal health care, free at the point of service for all the country's population, through a set of actions for health promotion and protection, disease prevention, cure, rehabilitation and palliative care. The SNS model is characterized by universality and accessibility (financed by national income taxes);

decentralization; comprehensive, continuous, people-centred care; PHC as the cornerstone and first point of patient contact with the system; high-quality services; and a high level of professional specialization.

The Ministry of Health is responsible for planning and regulation, and it oversees national regulatory authorities, including the Spanish Agency of Medicines and Medical Products. However, the administration, operational management, resource allocation, purchasing and provision of the SNS are fully devolved to the country's 17 autonomous communities, which coordinate their work through the Interterritorial Council. Resources are allocated with an eye towards improving equity, such that less prosperous regions dedicate the highest percentage of their GDP to health. The decentralized model fosters innovation and favours responsiveness to population needs on the one hand, but it entails variations in service models on the other.

Health and health equity in Spain

Health system performance

Spain is performing relatively well on the SDGs, ranking 14 out of 166 countries in 2024 (11). It has met SDG5 (gender equality), where its performance is particularly strong, and has met all but two targets for SDG3 (health and well-being), with the interregional gap in life expectancy constituting the only one where significant challenges remain (Annex 1). Moreover, Spain is committed to providing UHC (UHC index 85/100 in 2021 (11)), and it is fully compliant with the 2005 International Health Regulations, although the pandemic revealed gaps in preparedness and response capacities, which the government has since been actively working to fill. Regarding promotion of health and well-being, data show fairly stable trends on key behavioural risk factors, along with differences by region and according to educational attainment (Table 1) (9,12,13), indicating there is still more work to be done.

Table 1. Prevalence of key behavioural risk factors in Spain (% population), 2021 or latest year available

Risk factor	Total	Sex		Educational attainment			Interregional variation
		Male	Female	Low	Inter.	High	
Obesity in adults (18+ years)	16.0	16.5	15.5	21.5	13.0	10.1	6.8–19.7
Sedentarism in adults (18+ years)	36.4	32.3	40.3	45.5	31.8	24.6	20.9–61.5
Daily intake of fruits and vegetables	76.5	72.3	80.4	74.2	76.6	80.1	21.5–86.4
Current tobacco use, 15+ years ^a	28.4	29.4	27.5	–	–	–	15.2–25.3
Prevalence of cannabis smoking in adolescents (14–18 years)	21.8	22.5	21.0	24.1	24.2	21.5	9.8–31.4
Alcohol intake in previous year	65.5	74.6	56.8	–	–	–	32.9–77.9
Binge drinking in previous year	6.0	8.5	3.6	5.3	19.4	12.2	1.0–17.0

Source: SNS Annual Report 2022 (12) or 2023 (9), unless otherwise indicated.

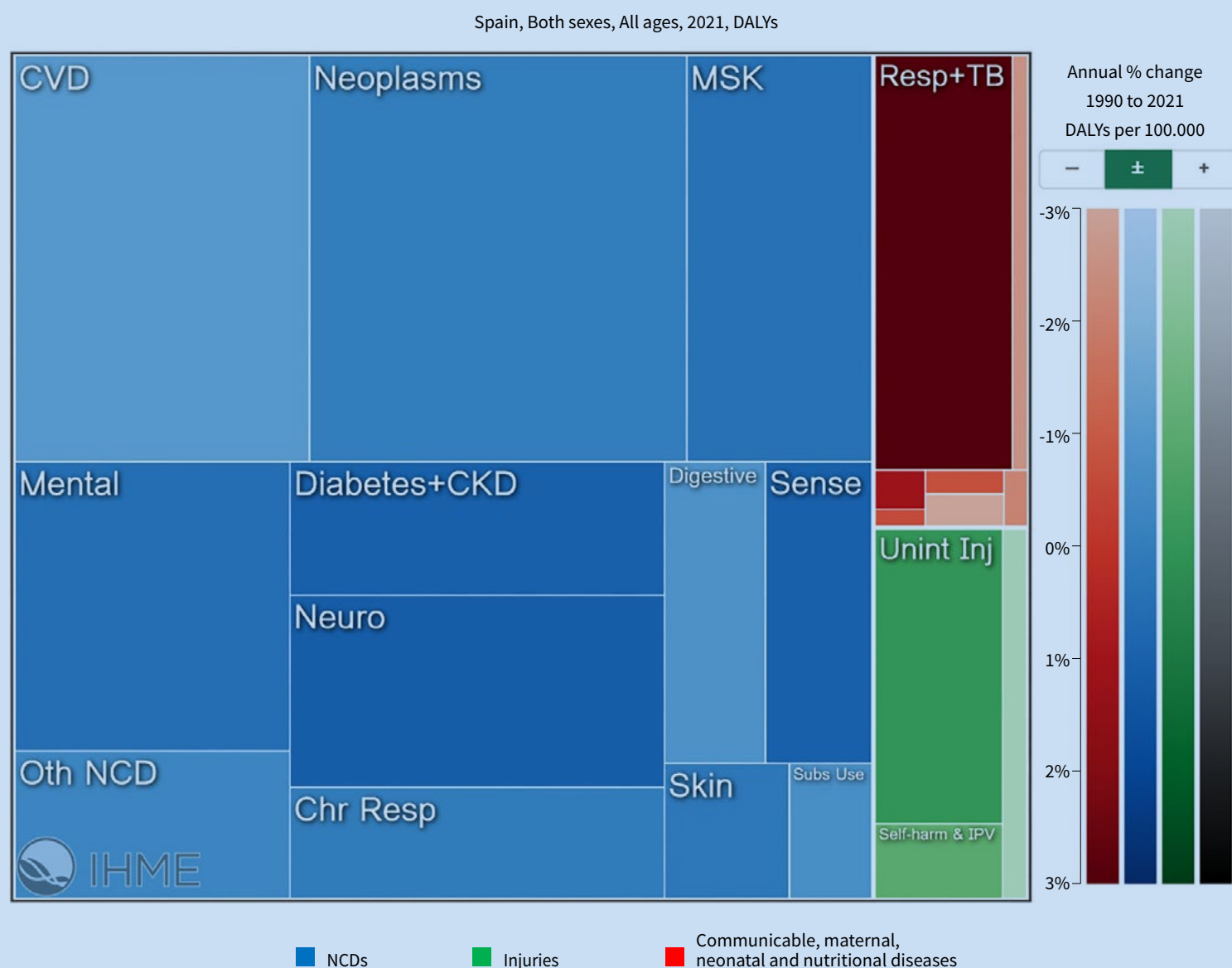
^a Age-standardized estimates for total population and by sex (2022 estimates from WHO) (13).

All in all, the SNS presents very good performance indicators: compared to the European Union average, Spain shows better healthy life expectancy at age 65, lower avoidable mortality, higher vaccination rates in infants and for influenza and coronavirus disease (COVID-19) in older people, higher breast and cervical cancer screening rates, and lower unmet medical needs (14). Other health system trends pose more concerns, particularly regarding the health workforce, which is under intense pressure and faces shortages in core areas, including nursing and PHC.

Disease burden and inequities

Broadly speaking, the burden of disease is dominated by noncommunicable diseases (NCDs), especially cardiovascular disease and cancer (Fig. 1), which together are responsible for over half of all deaths. However, there is also a high and rising prevalence of mental health disorders, affecting 34% of the population in 2024 (4), along with a steady rise in age-related neurological diseases like Alzheimer's.

Fig. 1. Distribution of burden of disease in Spain, 2019 (disability-adjusted life years).



Notes: CKD: chronic kidney disease; Chr Resp: chronic respiratory diseases; CVD: cardiovascular disease; DALYs: disability-adjusted life year; IPV: intimate partner violence; MSK: musculoskeletal disorders; Resp: respiratory diseases; TB: tuberculosis; Unint Inj: unintentional injury.

Source: (15).

Regarding health inequities, the *Spanish Public Health Strategy 2022 (16)* reviews key fault lines and health determinants in Spain. The main population characteristics covered are age, sex and gender, and sexual orientation and gender identity. The Strategy also acknowledges the vulnerability of highly marginalized subgroups, including the Roma people, immigrants (especially undocumented immigrants) and unhoused people, although there are fewer data on health indicators for these hard-to-reach groups. On the other hand, behavioural and environmental determinants are comprehensively reviewed, including nutrition; physical activity and sedentarism; tobacco, alcohol, illicit drugs and other addictions; sexual practices; climate-related determinants; exposure to pollution and environmental pathogens; socioeconomic and cultural determinants; school, community and occupational environments; health system characteristics and interregional differences; and globalization.

National health policies and plans

The Ministry of Health in Spain has a national strategic vision that will guide its work over the coming years both within the Spanish state and at a European and global level. Through it, the Ministry of Health in Spain will work to achieve four overarching objectives, underpinned transversally by a robust engagement with approaches that take into account the interdependence between human, animal and environmental health, and which promote gender equality, health equity and human rights:

1. **Recover pride in public health care.** The systemic shock caused by the COVID-19 pandemic, in both patients and the health workforce, has underlined the urgency of renewing the SNS's commitment to high-quality, responsive services and purposeful, rewarding working environments.
2. **Adapt healthcare to the 21st century.** The changing social, health and economic environment demand an increased focus on mental health and other NCDs; the climate crisis and emergency preparedness; the adoption of new digital and other health technologies; and a transformation of service models, beginning with PHC.
3. **Expand the coverage of the national health system.** Reaffirming its commitment to UHC, the Ministry of Health aims to expand the reach of health services to vulnerable populations and to enlarge and reinforce the portfolio of services to better cover population needs.
4. **Strengthen the Ministry of Health.** Reinforcing governance mechanisms to ensure efficiency and accountability, while strengthening the Ministry of Health's capabilities to advocate for intersectoral policies, will ultimately serve to improve population health.

Numerous national health plans and strategies are already in place (17). Current plans include the *Public Health Strategy 2022 (16)*, *Public Health Surveillance Strategy 2022 (18)*, the *Health and Environment Strategic Plan 2022–2026 (19)*, the *Strategic Framework for Primary and Community Care 2019 (20)*, the *Mental Health Strategy 2022–2026 (21)*, and the *Digital Health Strategy 2021–2026 (22)*, among others. The European Union-funded Recovery, Transformation and Resilience Plan (23), with a total budget of €163 billion, also includes a line of action to strengthen the SNS and science and innovation: 39.7% of the total budget will go towards the green transition and 28.2% towards the digital transition. These plans are fully in line with the GPW14 (1), EPW (2) and the PAHO Strategic Plan (3).

Global health and cooperation: Spain's approach and contributions

Spain's approach to international cooperation is currently undergoing a major reform and expansion following passage of the 2023 Law on Cooperation for Sustainable Development and Global Solidarity (24). The forthcoming VI Master Plan for Spanish Cooperation has been conceived to better position Spain as a leader in global health and development, boost implementation of the 2030 Agenda, and strengthen bilateral and multilateral partnerships in order to eradicate poverty and promote prosperity.

To support these goals, Spain will more than double its overall contributions to global development, from 0.3% of gross national income (US\$ 4.3 billion) in 2022 to 0.7% by 2030. Under the oversight of the State Secretariat for International Cooperation (part of the Ministry of Foreign Affairs, European Union and Cooperation), the Cooperation Council and its constituent bodies are working to reform the Spanish Agency for International Development Cooperation (AECID), draft Royal Decrees on grants and on the Spanish Fund for Sustainable Development, and reach a consensus on a budget coherent with the expanded development aid. Priority regions for increased cooperation are Latin America and the Caribbean; Asia; North Africa; the Middle East; and sub-Saharan Africa, in particular the countries of West Africa and the Sahel, as well as Portuguese- and Spanish-speaking countries.

In the same line, one of Spain's priorities is to finalize development of a new global health strategy, with the ultimate aim of promoting UHC and other SDG3 targets worldwide. The strategy is grounded in support for strong, multilateral health governance, led by WHO under the United Nations system, along with bilateral partnerships coherent with global frameworks and aimed at strengthening national health systems and services in individual countries.

Ongoing collaboration between Spain and WHO

Spain and WHO maintain numerous lines of ongoing cooperation; these are based on the national health agenda but support – and are supported by – the WHO GPW (1) and EPW (2) and the PAHO Strategic Plan (3). Spain provides WHO with technical assistance on WHO programmes, contributes to global initiatives with financial and in-kind resources, participates in WHO governance bodies and hosts 11 collaborating centres (Annex 2). As one of the two non-American observer states in PAHO (which functions as the WHO Regional Office for the region), Spain is also a major strategic partner in global health in the Americas. Taken together, these roles strengthen and amplify Spain's global health leadership, while also providing opportunities for Spanish policy-makers and experts to exchange best practices with countries facing similar challenges, cooperate on cross-border threats and tap into the deep well of scientific evidence, normative guidance and policy toolkits that WHO produces in service to its Member States.

The main channels of collaboration are briefly described below.

WHO Governing Bodies

Spain plays an active role in fostering accountability and efficiency in WHO operations, including as a member of the Standing Committee of the Regional Committee from 2022–2024. Further, it has presented its candidature for the WHO Executive Board in 2025.

Financial contributions

Spain is a major donor to WHO, contributing US\$ 43.8 million during the 2022–2023 biennium, most of which (76.3%) went to WHO headquarters in Geneva and to the African Region (19.3%). Assessed contributions accounted for around three-quarters of this amount, while specified contributions (mainly to respond to acute health emergencies) were the preferred channel for voluntary contributions. In addition, Spain is major donor to PAHO, contributing US\$ 10.8 million in voluntary contributions during the 2022–2023 biennium, most of which (53.6%) went to respond to outbreaks and health emergencies.

WHO collaborating centres

Among Spain's 11 WHO collaborating centres (Annex 2), several focus on priority topics of collaboration, for example the Andalusia School of Public Health (PHC), the Autonomous University of Madrid (mental health), the Catalan Institute of Oncology (tobacco control), the National Transplant Organization (organ transplantation), and the Barcelona Institute for Global Health (malaria control).

Technical networks

Spanish experts and authorities collaborate regionally and globally in an array of technical groups and networks coordinated by WHO, whether as Member State representatives, representatives of municipal or regional governments, or as academics affiliated with WHO through their institutions. A non-exhaustive list of these networks is presented in Annex 3.

Other key lines of collaboration

WHO and Spain also collaborate on specific technical programmes and initiatives related to overlapping global, regional and/or domestic priorities.

- **PHC.** June 2024 saw the inauguration of WHO's third PHC Demonstration Platform in the region of Asturias, opening up opportunities for other Member States to learn about Spain's model, while simultaneously raising awareness of the central role of PHC within Spain and increasing international recognition of its high quality and professionalization.
- **Organ transplantation.** Spain's National Transplant Organization works closely with WHO, managing the Global Observatory on Donation and Transplantation, combatting human organ trafficking and fully financing the WHO transplantation programme. In the PAHO region, Spain leads the Ibero-American Network/Council on Donation and Transplantation, through which it contributed to the development of a strategy and action plan on organ, tissue and cell transplants for the 2019–2030 period (25). Likewise, at the Seventy-seventh World Health Assembly, Spain was instrumental in the development and adoption of a global resolution, *Increasing availability, ethical access and oversight of transplantation of human cells, tissues and organs* (26).
- **Elimination of cervical cancer.** AECID partners with PAHO to encourage and stimulate political commitments and investments, as well as to promote the exchange of knowledge and experience for the elimination of cervical cancer. In alignment with the *WHO Global Strategy to Accelerate the Elimination of Cervical Cancer as a Public Health Problem* (27), AECID and PAHO are working to build technical capacity to advance cervical cancer elimination in low- and middle-income countries in the Americas.

- **Mental health.** Specific lines of joint work include collaboration on the WHO QualityRights initiative, membership in the Pan-European Mental Health Coalition Flagship Initiative, and participation in virtual meetings to advance ongoing joint work on mental health issues. The WHO Collaborating Centre for Mental Health Services Research and Training (Autonomous University of Madrid) also serves as the lead expert body supporting the WHO Regional Office for Europe in designing and implementing a survey on the mental health of the health workforce.
- **Emergency preparedness and response.** Spain is working with WHO on the development of the Epidemic Intelligence from Open Sources tool to support risk detection. It has an active role in the network of International Health Regulation focal points and participates in the Pan-European Network for Disease Control, the Emergency Medical Teams (EMT) Initiative, and the Regional European Steering Group for EMT Initiative. Beyond the European Region, Spain supports the EMT Americas Initiative and contributes to the Global Outbreak Alert and Response Network.
- **Childhood obesity.** Spain is a longstanding member of the WHO Childhood Obesity Surveillance Initiative: in 2023, the country hosted a high-level meeting on childhood obesity in Mallorca, and in 2024, it participated in the WHO policy dialogue for southern European countries. As one of the frontrunner countries for the *WHO acceleration plan to stop obesity* (28), Spain is a global leader in the implementation of comprehensive approaches to reduce obesity (including through marketing regulations) and improve access to obesity management services.
- **Tobacco control.** Through its WHO Collaborating Centre (Unit of Tobacco Control, Catalan Institute of Oncology), Spain is actively involved in tobacco control efforts at both regional and global levels. This work includes organizing workshops, seminars and training courses; contributing to the tobacco control research agenda; and hosting biennial international symposia on current topics of interest in tobacco control. Furthermore, Spain is a Party to the WHO Framework Convention on Tobacco Control and performs regional coordination functions.
- **Health in the Americas.** AECID's strategic partnership with PAHO was articulated in a Memorandum of Understanding, signed in 2023 (29). In it, AECID committed nearly €1.5 million in aid toward obstetric and neonatal emergencies, cervical cancer elimination, information systems for health, sexual and reproductive health, human resources for health, and clinical capacity-building for disasters and pandemics.
- **Equity and gender equality.** AECID partners with PAHO to strengthen Member State capacities and knowledge around gender- and ethnicity-related health inequalities, including through the PAHO's first report on Masculinity and Health (30) and its *Strategy and Plan of Action on Ethnicity and Health 2019-2025* (31). The Ministry of Health, through the Women's Health Observatory, is also actively collaborating with WHO in international fora to develop new policies and indicators for gender equality in health. Looking forward, Spain is organizing an international event to commemorate the 30th anniversary of the Beijing Declaration and to launch a Global Agenda on Women and Health in cooperation with the WHO Gender Unit (WHO headquarters) and the WHO European Office for Investment for Health and Development.
- **Violence Against Women.** In line with Spain's feminist foreign policy, AECID partners with PAHO to strengthen the health system's response to violence against women in Latin America and to improve access to health care for adolescent girls and adolescent survivors of sexual violence.
- **Medicines.** Spain is an active member of the WHO Regional Office for Europe's Access to Novel Medicines Platform and its working group on sustainability. Moreover, it has supported the development of three key project proposals on horizon scanning, principles for managed entry agreements and precision medicine for cancer and rare diseases.

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- **HIV and stigma.** Spain regularly participates in the annual Joint United Nations Programme on HIV and AIDS (UNAIDS) Global AIDS Conferences, and in September 2023, the country hosted the WHO Regional Office for Europe, UNAIDS, the European Centre for Disease Prevention and Control, and other key stakeholders for a series of high-level meetings focused on galvanizing regional responses to HIV/AIDS, promoting the human rights dimension, and reinforcing WHO European Region Member States' commitment to achieving AIDS and Zero discrimination goals.
 - **Tuberculosis (TB).** Spain supports the commitments made at the United Nations high-level meeting on TB (32), including universal access to TB services for special populations, elimination of all forms of discrimination, community participation, access to drug-resistant TB treatment, integrated approaches to TB and HIV, and investment in TB research, with a special interest in the TB Vaccine Accelerator.
 - **Malaria.** Since 2000, the Carlos III Health Institute's National Centre of Tropical Medicine in Madrid has been collaborating with Equatorial Guinea on the country's Malaria Control Programme, in cooperation with the WHO local malaria section.
 - **Neglected tropical diseases (NTDs).** The Carlos III Health Institute, through the National Centre of Tropical Medicine, provides technical and financial assistance to national NTD programmes in Africa. These programmes also receive technical support from WHO's Expanded Special Project for Elimination of NTDs. Activities have included the performance of onchocerciasis surveys and mass treatment for schistosomiasis. Furthermore, Spain is a member of the Joint Coordinating Board of the Tropical Diseases Programme, sharing a constituency with Panama.
 - **Health-care fraud and corruption.** Spain collaborates with the WHO to combat health-care fraud and corruption. In 2024, the Ministry of Health hosted a symposium on health-care corruption, co-organized with the WHO Regional Office for Europe and the European Observatory on Health Systems and Policies, with participation from the European Healthcare Fraud & Corruption Network. Nationally, Spain has established the Observatory against Healthcare Fraud and Corruption and plans to expand cooperation with European and international bodies.



Strategic priorities

In the context of the myriad channels of ongoing collaboration between Spain and WHO, the CCS provides an opportunity to optimize alignment between Spain's domestic and global health agenda, the GPW14, the EPW and the PAHO Strategic Plan. Five strategic priorities have been designated: these respond to WHO's added value in supporting Spain's domestic health objectives and maximizing the impact of its contributions and leadership abroad.

1. Reinforce the foundations of **UHC** through health workforce planning and team care, transformation of PHC and increased access to complex services.
2. Address **NCDs** through an evidence-based, intersectoral, community- and person-centred approach.
3. Strengthen **multilateral approaches to health emergency preparedness and response**.
4. Accelerate a just transition towards a **climate-resilient, sustainable health system and society**.
5. Strengthen the control of **infectious diseases** at a global level.

In addition, a set of **cross-cutting principles** should permeate all joint actions:

Health and human rights. Collaboration between Spain and WHO must safeguard not only the right to health, but also other rights enshrined in the Universal Declaration on Human Rights.

Equity. A core principle of the SNS, health equity is a key consideration when developing, implementing, and monitoring policy interventions to explicitly address the social determinants of health, including inequities tied to socioeconomic conditions, geography, disability, sexuality and gender identity, racism and migrant status.

One Health. The interdependence between human, animal and environmental health requires concerted action by multiple sectors, disciplines and communities.

Gender equality. In the framework of Spain's feminist foreign policy, the Spanish CCS must be feminist, recognizing the need for women's roles and leadership in the health and care sector as well as the challenges that women worldwide still face in realizing their right to health.

Each priority is composed of several strategic deliverables, which are captured in a high-level theory of change (Fig. 2), whereas Table 2 maps the CCS against the GPW14 (1), EPW (2), and the Spanish Public Health Strategy 2022 (16) (Table 2).

Fig. 2. High-level theory of change for the CCS between Spain and WHO

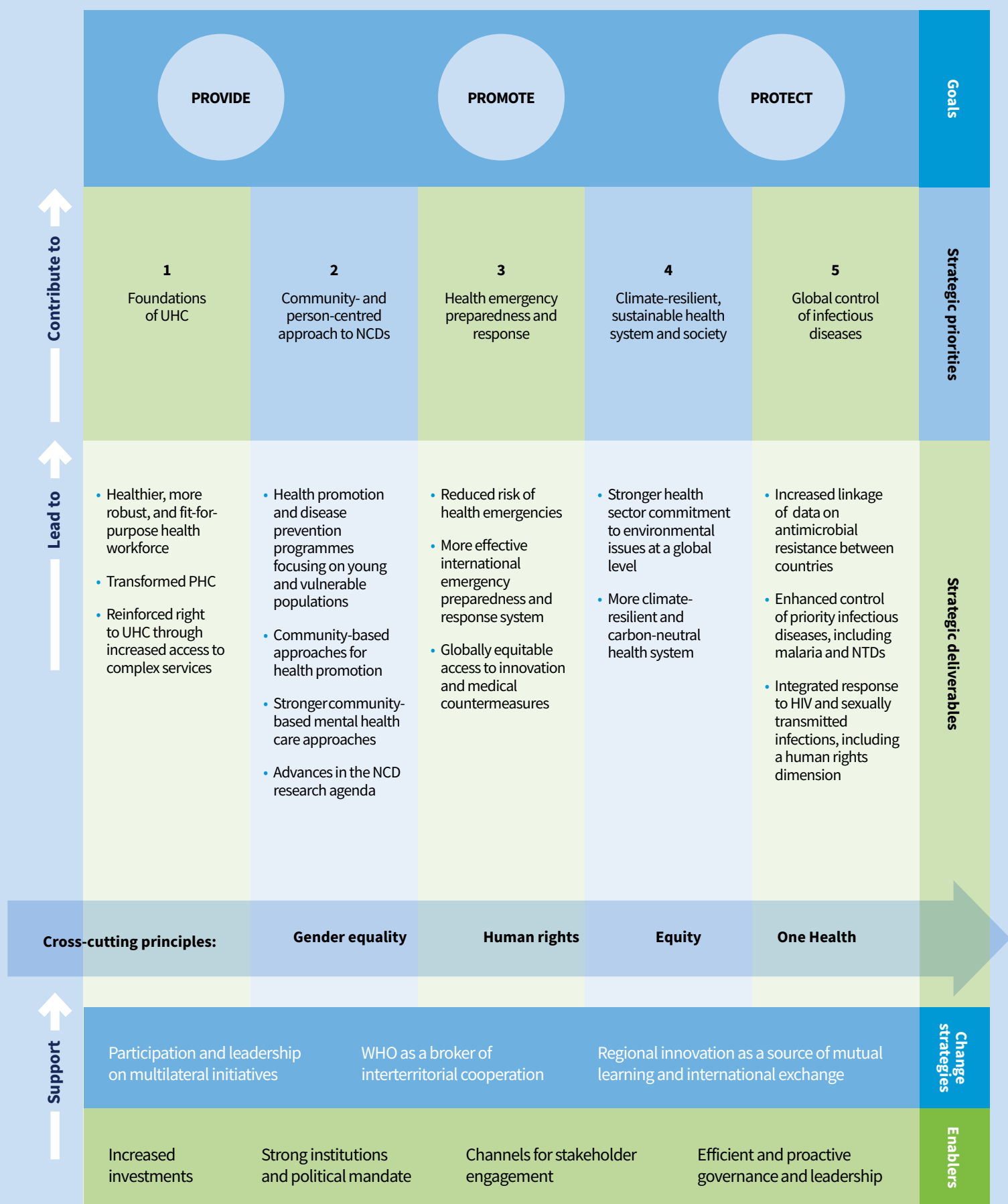


Table 2. Links between the CCS for WHO and Spain and the GPW14, EPW and the Spanish Public Health Strategy

Spanish Public Health Strategy work stream	GPW14 objectives	EPW core priorities
CCS Strategic Priority 1. Reinforce the foundations of UHC through health workforce planning and team care, transformation of PHC and increased access to complex services		
- Strengthen public health governance, capacities, systems and advocacy	- Advance the PHC approach and essential health system capacities for UHC - Improve health service coverage and financial protection to address inequity and gender inequalities	Moving towards UHC
CCS Strategic Priority 2. Address NCDs through an evidence-based, intersectoral, community-and person-centred approach		
- Improve population health through promotion of healthy lifestyles; disease prevention; and healthy, safe and sustainable environments - Promote health and health equity across the life course - Update disease surveillance systems to strengthen preparedness and response	- Address health determinants and the root causes of ill health in key policies across sectors - Prevent, mitigate and prepare for risks to health from all hazards - Rapidly detect and sustain an effective response to all health emergencies	Promoting health and well-being Protecting against health emergencies
CCS Strategic Priority 3. Strengthen multilateral approaches to health emergency preparedness and response		
- Update disease surveillance systems to strengthen preparedness and response	- Prevent, mitigate and prepare for risks to health from all hazards - Rapidly detect and sustain an effective response to all health emergencies	Protecting against health emergencies
CCS Strategic Priority 4. Accelerate a just transition towards a climate-resilient, sustainable health system and society		
- Improve population health through promotion of healthy lifestyles; disease prevention; and healthy, safe and sustainable environments	- Respond to climate change, an escalating health threat in the 21st century	Promoting health and well-being
CCS Strategic Priority 5. Strengthen the control of infectious diseases at a global level		
- Update disease surveillance systems to strengthen preparedness and response - Improve population health through promotion of healthy lifestyles; disease prevention; and healthy, safe, and sustainable environments	- Advance the PHC approach and essential health system capacities for UHC - Prevent, mitigate and prepare for risks to health from all hazards	Moving towards UHC

Strategic priority 1. Reinforce the foundations of UHC through health workforce planning and team care, transformation of PHC, and increased access to complex services

UHC is a central pillar of both the SNS and the 2030 Agenda. Effectively achieving UHC requires a systems approach, supported by high-quality PHC and a strong health workforce as indispensable pillars of health system efficiency, sustainability and resilience. The commitment to UHC also implies a commitment to equity, with the understanding that everyone should be able to freely access the same high-quality, essential health services, regardless of their ability to pay, their place of residence, or the complexity of their condition.

Healthier, more robust, and fit-for-purpose health workforce

Over the coming years, WHO will work with Spain to ensure a healthy, satisfied and well-resourced workforce as the backbone of a strong health system. In Spain, technical collaboration will focus on developing a comprehensive strategy and planning model for human resources for health, with a focus on PHC, together with nursing and midwifery. At the time of writing, the SNS was developing its Roadmap and Strategic Framework on Care, based on the WHO *Global strategic directions for nursing and midwifery* (33). The Roadmap will include actions for nurse specialists and midwives and an innovation mapping exercise, to be overseen by Spain's newly appointed Chief Nursing and Midwifery Officer and Director of Nursing and Midwifery. It will also be coherent with the WHO Regional Office for Europe's *Framework for action on the health and care workforce in the WHO European Region 2023–2030* (34). International collaboration will be articulated through participation in European initiatives to address the underlying reasons for health workforce shortages, including burnout and inadequate working conditions.

Transformed PHC

While the SNS remains an international reference in PHC, Spain's autonomous communities have responded unevenly to emerging challenges. Given the shared values and common model across the country, the innovations introduced in some regions may inspire the necessary transformation of PHC elsewhere, helping to ensure that services throughout the country remain high-quality, person-centred and equitable. The Ministry of Health is aiming to introduce more flexibility in the criteria for defining health-care areas and basic health zones, considering factors like the density of the population and health-care workforce and the social determinants of health. Other priorities include optimizing coordination between PHC, social care and community services, along with fully implementing plans and initiatives brought forward during the COVID-19 pandemic (35).

For its part, WHO will keep working closely with Spain in the context of the WHO PHC Demonstration Platform in Spain. This platform can enable other countries from the WHO European Region and beyond to apply lessons from the Spanish PHC model to their own systems, while allowing Spain to engage in international exchanges with other countries that are prioritizing PHC reforms. Additionally, WHO remains ready to provide ad hoc technical assistance to Spain around key issues related to PHC policy and practice.

Reinforced right to UHC through increased access to complex services

UHC does not stop at the primary level – patients also need to be able to access highly specialized services, including in areas like rare diseases and organ transplantation. Thus, WHO and Spain, in collaboration with other Member States, will work together to promote access to health services for rare diseases worldwide through a specific resolution to be submitted to the World Health Assembly. Additionally, WHO and Spain will collaborate to implement the Spanish-led resolution on organ transplantation adopted by the Seventy-seventh World Health Assembly (26). This includes the development of a Global Transplant Strategy and different regional strategies and plans to help other countries integrate donation and transplantation of cells, tissues and organs into their health-care system; promote the Guiding Principles of WHO; and increase availability and equitable access to transplants.

Strategic priority 2. Address NCDs through an evidence-based, intersectoral, community- and person-centred approach

Today, NCDs stand out as the most important cause of mortality and morbidity in Spain, as in the world. Their multifactorial aetiology, social determinants and slow course require intersectoral, whole-of-society approaches that challenge health systems to foster horizontal leadership, community participation, patient empowerment and regulatory protection. WHO has developed a 5×5 approach that focuses on the five main NCD groups (cardiovascular diseases, cancer, diabetes, chronic respiratory diseases and mental health), together with the five most important shared risk factors (tobacco and related products, physical inactivity, alcohol, unhealthy diet and environmental pollution). In Spain, priority avenues for implementing this agenda include a focus on prevention in young people and vulnerable groups, advances in the NCD research agenda and community-based approaches.

Health promotion and disease prevention programmes focusing on young and vulnerable populations

The Ministry of Health is actively implementing different measures to tackle NCD risk factors in children and young people. These efforts include new legislation to reduce tobacco use (36) and alcohol consumption (37), the establishment of an Interministerial Committee to combat childhood obesity in Spain, implementation of the *National Strategic Plan for the Reduction of Childhood Obesity (2022–2030)* (38), and a *Cardiovascular Health Strategy* (39), among others. In the coming years, Spain will collaborate with WHO on this agenda by being a frontrunner in the *WHO acceleration plan to stop obesity* (28); achieving best-practice levels of tobacco control through full implementation of all MPOWER measures in Spain and promoting analogous tobacco control efforts in the European Region; participating in regional initiatives like the Youth Alcohol Network for the Evidence into Action Alcohol Project to reduce alcohol intake; and fostering patient participation and empowerment in those with chronic diseases.

Community-based approaches for health promotion

Health-related behaviours and outcomes throughout the life course are largely a function of a person's environment, social circles, living and working conditions, societal norms and gender roles, and access to healthy food and leisure activities. Intersectoral approaches are thus crucial for improving population health through cleaner, greener and more health-promoting cities and communities. In Spain, the Spanish Healthy Cities Network is a WHO-accredited national network affiliated with the WHO European Healthy Cities Network, while in the Americas, the Movement of Municipalities, Cities and Healthy Communities operates with similar goals. In the coming years, PAHO will work with Spain to facilitate the exchange of experiences and best practices. In addition, WHO and Spain will work together to enhance the health-promoting schools approach: a valuable avenue for reaching all children, including the most vulnerable, with early interventions in nutrition, physical activity and health literacy.

Stronger community-based mental health care approaches

The impacts of the COVID-19 pandemic laid bare the great unmet need for mental health services in European populations. To address this demand in Spain, the Mental Health Commissioner figure was created in 2024 to advance priority streams of work. In this regard, a new action plan on mental health is being developed: it will cover work-related mental suffering; mental health in adolescents and young people; deinstitutionalization; a rights-based approach to serious mental disorders; social prescribing; enhanced mental health information systems; and improved prescription practices for psychotropic drugs. Another action plan against suicide for 2025–2027 will focus on enhanced surveillance, responsible media interactions for reporting on suicide, identification of vulnerable populations and improved access to essential health services. It will also prioritize multisectoral collaboration and promote clinical and healing approaches with a strong emphasis on family and community support. The Suicide Prevention Health Observatory will serve as the national body for interagency coordination.

These objectives and priorities are fully aligned with WHO initiatives, including the *WHO European framework for action on mental health 2021–2025 (40)* and the Pan-European Mental Health Coalition. Spain is involved in the six priority areas of this Coalition and is actively engaged in line 4, the development of workplaces that promote and protect mental health and well-being. In the coming years, the two parties will work together through these and other multicountry forums to exchange best practices, provide mutual support and learning, and promote a human rights-based approach to mental health in Spain and beyond. Spain will also disseminate the WHO signature QualityRights e-training materials in Spanish to reach wider audiences.

Advances in the NCD research agenda

A large body of evidence already exists on NCD determinants and management approaches, but gaps in knowledge still remain. Over the life of the CCS, Spain and WHO will collaborate to generate evidence on emerging areas of research interest, such as food environments and the commercial determinants of health; to improve surveillance tools and data; and to test the feasibility of assessing the prevalence of depression and anxiety across Europe. The GPW 14 results framework (41) outlines the next steps (including a section on joint accountability for WHO and Member States) required to achieve public health impact. The research agenda can generate outcomes for each of these steps and develop context-specific tools to measure them, with an emphasis on identifying facilitators and barriers for NCD policy implementation.

Strategic priority 3.

Strengthen multilateral approaches to health emergency preparedness and response

The COVID-19 pandemic underscored the vital need for strong international systems and instruments to share data, build surveillance and response capacity, ensure equal access to medical countermeasures, and tackle global and cross-border threats to health. Outbreaks and health emergencies can happen at any time, anywhere, and they do not respect borders. Therefore, all countries must be prepared and equipped to effectively anticipate, respond to, and recover from the impacts of likely, imminent or actual threats to health. Over the next several years, Spain and WHO will collaborate to strengthen global systems, while also leveraging these activities to build capacity within the Spanish state.

Reduced risk of health emergencies

With respect to reducing risk and mitigating the impact of health emergencies, Spain follows an all-hazards approach, using its own resources and those of the European Union, while WHO mainly contributes through the establishment of networks to facilitate the exchange of good practices and coordination. For example, Spain is a member of the WHO Regional Office for Europe's Pan-European Network for Disease Control as well as the Global Outbreak Alert and Response Network. Moreover, Spain is working with WHO on the development of the Epidemic Intelligence from Open Sources tool to facilitate risk detection. WHO can also support Spain in the framework of the International Health Regulations and the WHO Emergencies Programme.

More effective international emergency preparedness and response systems

The Coordination Centre for Health Alerts and Emergencies, under the authority of the Ministry of Health, is the Spanish body responsible for coordinating emergency preparedness and response activities for public health threats. In the coming years, it will work with WHO to build up multilateral systems in this field, for instance through the strengthening of regional and global networks for information exchange in line with the One Health approach, participation in working groups to develop collective tools and approaches to common threats, capacity-building for rapid response and surge capacities, and the provision of technical and financial assistance to low-resource countries to shore up their preparedness and response capacities. Spain will continue to strengthen the EMT Initiative at strategic and technical levels by participating in the Regional European EMT Steering Group, contributing to the development and implementation of the *Emergency Medical Teams regional action plan 2024–2030* (42), and strengthening its own EMT capacities at national level.

These activities, together with the continuous development of the WHO Regional Office for Europe's preparedness framework – Preparedness 2.0 – will add value to the Coordination Centre for Health Alerts and Emergencies' work within Spain, ensuring that Spain's emergency preparedness and response systems contribute to – and benefit from – global efforts to keep populations safe from health emergencies.

Globally equitable access to innovation and medical countermeasures

Spain is aligned with WHO regarding the imperative to ensure that all countries have adequate access to health products in line with the *WHO Global strategy and plan of action on public health, innovation and intellectual property* (43). In this regard, the two parties will work together to promote a more effective and equitable model of innovation and access to essential medicines, vaccines, treatments and diagnostics. Spain will also help WHO to mobilize the political will necessary to finalize the negotiation, development and implementation of the future accord to prevent, prepare and respond to pandemics (Pandemic Accord).

Strategic priority 4. Accelerate a just transition towards a climate-resilient, sustainable health system and society

The dramatic health impacts caused by the triple crisis of climate change, pollution and biodiversity loss highlight the urgency of integrating the climate dimension into health systems, both to adequately address risks and to reorient services to be more climate-resilient and sustainable. Since 1991, the European Centre for Environment and Health (ECEH) has taken the lead role in the WHO European Region in this field. And as the effects of the climate crisis have intensified, both WHO and individual Member States have stepped up their efforts to adjust systems to a new climate reality. In Spain, the Ministry of Health has adopted the *Strategic Health and Environment Plan 2022-2026* (44) and through the Health and Climate Change Observatory, it is promoting a roadmap and clear lines of action to galvanize a just transition towards healthier and more resilient societies. Over the course of the CCS, the two parties will work together both multilaterally and bilaterally to build resilience and sustainability into the foundations of the health system.

Stronger health sector commitment to environmental issues at a regional and global level

In the European Region, the WHO Regional Office for Europe is leading joint action on climate change and health, with the 2023 Budapest Declaration (45) representing the primary vehicle for articulating Member States' commitments to a thriving planet and sustainable, climate-conscious health systems. Through the CCS, Spain is invited to join and engage on this agenda, specifically through the European Environment and Health Process (EHP) Partnership for Health Sector Climate Action and the Working Group on Health in Climate Change, established under the European Environment and Health Task Force.

At the global level, WHO serves as the secretariat to the Alliance for Transformative Action on Climate and Health (ATACH), committed to building climate-resilient and low-carbon sustainable health systems and to other climate change and health priorities. Spain joined ATACH at the 2021 United Nations Climate Change Conference and hosted its first face-to-face meeting in March 2024 in Madrid. From 2025, AECID will be represented in the ATACH Steering Group as a key donor. For the CCS, Spain will continue to support WHO in such global forums and initiatives, working to integrate health considerations into national adaptation plans and nationally determined contributions; taking an active part in the health-related dialogues held during the United Nations Climate Change Conferences; sharing its experience in calculating the carbon footprint of hospitals with other countries and conducting trainings for their health workforce to do the same (46); and supporting ATACH from a European regional perspective, for example through the EHP Partnership.

Spain will also actively engage in the regional Working Group on Health in Climate Change, contributing its expertise emerging from the Health and Climate Change Observatory. Furthermore, Spain will support WHO by providing peer-to-peer technical support to other countries in ATACH for the implementation of their commitments and contribute to interregional health programmes in environmental health, such as the transatlantic dialogues on climate and health. In this sense, an important area of collaboration with PAHO will be in guaranteeing the sanitary quality of water and the development of Water Sanitation Plans.

A climate-resilient and carbon-neutral SNS

Spain will intensify its collaboration with the ECEH and its climate change and health networks, including the EHP Partnership and the Health in Climate Change working group, as a path to achieving a carbon-neutral SNS by 2050. Spain will also closely collaborate with the ECEH and peer Member States in the European Region to further develop and implement the WHO Regional Office for Europe's guidance on heat-health action plans in Spain.

Strategic priority 5. Strengthen the control of infectious diseases at a global level

The combined effects of climate change and globalization are blurring the spatial boundaries once taken for granted in worldwide epidemiological geography. A warming planet has favoured the spread of disease vectors like the *Aedes* mosquito as well as the propagation of antimicrobial-resistant pathogens. Surging migration and international tourism are increasing the risks of imported infectious diseases, including NTDs and TB, while the reduced threat of HIV/AIDS has led to relaxed societal norms around safe sex and the prevention of other sexually transmitted infections (STIs). In this context, global health solidarity represents more than coherence with the principles of health equity and justice; it also effectively protects health at home. For Spain, priority areas of work to achieve global control of infectious diseases include the digitalization of antimicrobial resistance (AMR) surveillance systems on a global scale, together with enhanced global control of priority infectious diseases, including malaria, NTDs, HIV and other STIs.

Strengthened AMR surveillance and data integration across the European Region

AMR is a global challenge, and prioritizing preventive measures to swiftly detect and contain transmission is crucial. To that end, all countries need to improve their AMR information systems to ensure effective data integration and global sharing of resistance patterns. Establishing standardized and compatible surveillance systems is essential for building a holistic picture of AMR across regions. Achieving comprehensive, cross-country data integration requires strengthening national capacities to generate robust and reliable data. In many parts of the WHO European Region, surveillance systems are still inadequate, leading to gaps in understanding resistance patterns, putting patient care in danger. Addressing this involves supporting countries in developing effective AMR surveillance systems, aligning laboratory practices with international standards, and strengthening data-sharing platforms including the European Antimicrobial Resistance Surveillance Network, the Central Asian and European Surveillance of Antimicrobial Resistance, and the

Global Antimicrobial Resistance and Use Surveillance System. Additionally, establishing international alert mechanisms is key to responding rapidly to emerging threats and ensuring timely containment.

In the coming years, Spain and WHO will work together to implement the *roadmap on antimicrobial resistance for the WHO European Region 2023–2030* (47). Domestic implementation of this roadmap is guided by the Spanish National Action Plan on AMR (2021–2024) (48), in full alignment with the regional strategy. The collaboration between WHO and Spain will focus on the enabling areas of the roadmap related to surveillance, stewardship, infection prevention and control, and laboratory strengthening and digital technology by improving AMR data linkages between countries, enhancing information flow and facilitating the implementation of containment measures as efficiently as possible. Through these efforts, the region will be better equipped to address AMR comprehensively and collectively.

Enhanced control of priority infectious diseases

Globalization and climate change are converging to make Spain more vulnerable to emerging and re-emerging communicable diseases, including leishmaniasis, Chagas, West Nile virus and dengue. The intensified interest in this field at a country level has resulted in the formation of a working group on multidisease elimination and prevention of the reestablishment of vector-borne diseases.

The CCS enables engagement on this agenda across national and regional borders. The WHO Regional Office for Europe and Spain, in dialogue with PAHO, will work to develop a framework for action in this direction, including setting up a validation process for multidisease elimination. This work can also tie into the introduction of the new TB vaccine in the European Region and joint working groups on vector-borne diseases.

In the WHO African Region, the Carlos III Health Institute will continue to collaborate with the WHO Department of Control of Neglected Tropical Diseases, the WHO Regional Office for Africa's Expanded Special Project for Elimination of Neglected Tropical Diseases, and the Global Network for the Elimination of Onchocerciasis. Spain will also continue to work on laboratory and control activities in Equatorial Guinea through a partnership with the country's Malaria Control Programme, the WHO Neglected Tropical Diseases Programme and the WHO local malaria section.

Integrated response to HIV and STIs, including a human rights dimension

Spain's *Strategic Plan for the Prevention and Control of HIV and STIs 2021–2030* (49) is aligned with the WHO global health sector strategies on HIV, viral hepatitis, and STIs and the *Regional action plans for ending AIDS and the epidemics of viral hepatitis and sexually transmitted infections 2022–2030* (50). Both propose a common vision to end the STI epidemic in an integrated manner, incorporating elements of innovation and integrated community-led service models for HIV prevention and testing, with a special emphasis on eliminating stigma and discrimination for key populations and people living with HIV. During the World Health Assembly in May 2024, Spain proposed a resolution urging countries to take bold measures to control STIs and to establish a "World STI Day", on which it is working jointly with WHO. Other areas of collaboration include efforts towards the validated triple elimination of vertical transmission of HIV, viral hepatitis B and syphilis; and exchange of best practices in scaling up HIV prevention through pre-exposure prophylaxis.

Implementing arrangements

Implementation of the CCS will span the period from 2024 to 2030. A separate work plan details how the CCS will be operationalized, including the strategic deliverables, key interventions and outcome indicators corresponding to each strategic priority along with the institutional actors responsible for implementation. The work plan is conceived as a “living” instrument, internally accessible to both WHO and the Spanish Ministry of Health, working in concert to deliver on the CCS. It will be updated on an ad hoc basis to reflect any relevant changes in institutional arrangements, resource availability and objectives.

At the same time, focal points will be appointed by both the Ministry of Health and the WHO Regional Office for Europe to ensure coordination and coherence with the strategic priorities and deliverables, prioritize the interventions with the potential for the greatest impact, and reinforce the integration of the cross-cutting principles (equity, human rights, gender equality and One Health) throughout all lines of strategic action.

Monitoring and evaluation

The work delivered under this CCS will be reported on through existing reporting processes (of both the Ministry of Health and WHO), and there will be regular stock-taking of progress as the workplan is delivered.

This pragmatic approach will be complemented by a yearly check-in between Ministry of Health and the WHO Regional Office for Europe. The Ministry of Health will liaise with other Spanish bodies, including AECID, to keep track of progress in implementation, while the Regional Office for Europe will communicate regularly with other levels of WHO, including WHO headquarters in Geneva and PAHO in the Americas.

The yearly check-in will serve as a more structured stock-take of progress, identify bottlenecks and guide adjustments to implementation mechanisms as needed, including through the consideration of resources and/or changes in the responsible actors to carry the work forward. The continued alignment between the CCS and Spain's national and global health agenda will also be considered, as will evolving priorities at a regional and global level.



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¹ All references were accessed 9 October 2024.

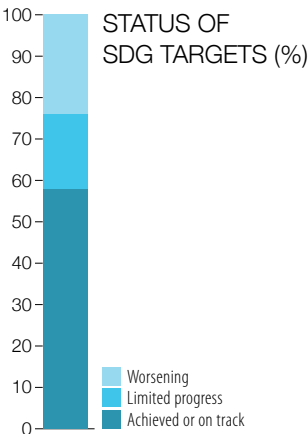
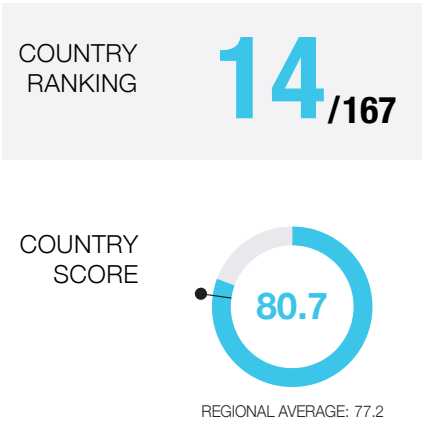
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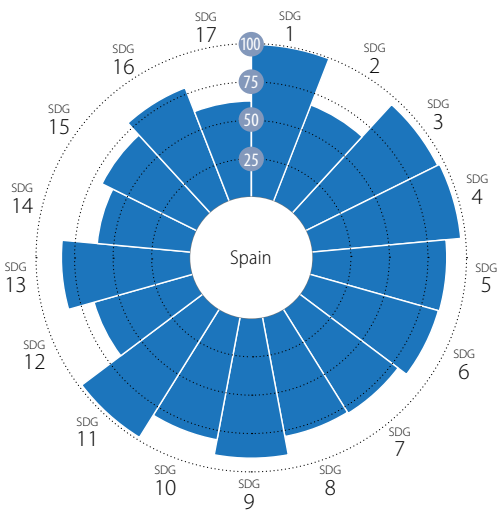


Annex 1. Sustainable Development Goal performance in Spain

OVERALL PERFORMANCE



AVERAGE PERFORMANCE BY SDG

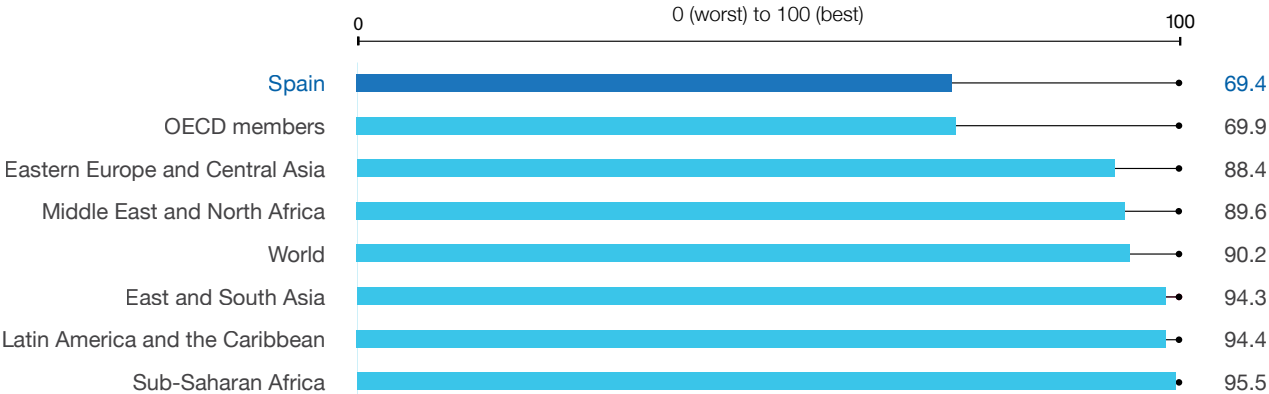


SDG DASHBOARDS AND TRENDS

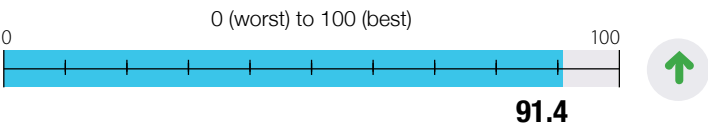


Note: The full title of each SDG is available here: <https://sdgs.un.org>

INTERNATIONAL SPILLOVER INDEX



STATISTICAL PERFORMANCE INDEX



MISSING DATA IN SDG INDEX

1%

Annex 2. WHO collaborating centres in Spain

Technical area	Centre	Parent institution	WHO responsible officer
WHO Regional Office for Europe-designated collaborating centres^a			
Mental Health Services Research and Training	Psychiatry Department, Medical School	Autonomous University of Madrid	Brandon Gray
Tobacco Control	Tobacco Control Unit	Catalan Institute of Oncology	Angela Ciobanu
Vaccine Safety	Pediatrics Department	Santiago de Compostela Clinical University Hospital	Liudmila Mosina
WHO headquarter-designated collaborating centres			
Primary Health Care	International Health Area	Andalusia School of Public Health	Sophie Genay-Dilautas
Malaria Control, Elimination and Eradication	Research Area	Barcelona Institute for Global Health	Elkhan Gasimov
Leishmaniasis	Unit for Leishmaniasis and Chagas Disease	Carlos III Health Institute	Jose Antonio Ruiz Postigo
Fascioliasis and Its Snail Vectors	Sanitary Parasitology Unit Pharmacy, Pharmaceutical Technology and Parasitology Department	University of Valencia	Denise Mupfasoni
Clinical Management of Leishmaniasis	National Reference Unit for Tropical Diseases, Department of Infectious Diseases	Ramon y Cajal University Hospital	Jose Antonio Ruiz-Postigo
Substance Use, Noncommunicable Diseases and Policy Impact	Subdirector General on Addictions, HIV, STD and Viral Hepatitis	Public Health Agency of Catalonia	Maria Neufeld
Global Monitoring of Palliative Care Development	Global Palliative Care Observatory Atlantes, Institute for Culture and Society	University of Navarra	Kathryn Patricia O'Neill
Donation and Transplantation		National Transplant Organization	Efstratios Chatzixiros

^a Data as of September 2024. For additional, up-to-date details, check: WHO Collaborating Centres Global Database [online database]. World Health Organization; 2024 (<https://apps.who.int/whocc/List.aspx?aCUQkcMSXrAlp8Oa6V7a8A==>).

Annex 3. WHO technical networks with participation from Spain

Spain participates in a number of WHO technical networks including:

- WHO Action Network on Measuring Population and Organizational Health Literacy;
- European Environment and Health Task Force;
- European Technical Advisory Group of Experts on Immunization;
- Pan-European Commission on Health and Sustainable Development;
- Regional and global influenza laboratory network;
- Regions for Health Network (Catalonia, Andalusia);
- Technical Advisory Group on the mental health impacts of COVID-19 in the WHO European Region;
- Healthy Cities Network (phase VII includes the Spanish National Health Cities Network and the following cities in Spain: Barcelona, L'Hospitalet de Llobregat, Sant Andreu de la Barca, Villanueva de la Cañada and Vitoria-Gasteiz);
- WHO Regional Office for Europe's Access to Novel Medicines Platform;
- WHO European Childhood Obesity Surveillance Initiative;
- WHO Acceleration Plan to Stop Obesity (frontrunner country for WHO European Region);
- WHO European Action Network on Reducing Marketing Pressure on Children;
- WHO European Health-Enhancing Physical Activity network;
- Network on Intersectoral Action and Social Participation for Health Equity in the Americas;
- Transport, Health and Environment Pan-European Programme;
- Pan-European Network for Disease Control;
- Pan-European Mental Health Coalition; and
- The Mental Health Coalition Flagship Initiative.

The WHO Regional Office for Europe

The World Health Organization (WHO) is a specialized agency of the United Nations created in 1948 with the primary responsibility for international health matters and public health. The WHO Regional Office for Europe is one of six regional offices throughout the world, each with its own programme geared to the particular health conditions of the countries it serves.

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Belgium	Kazakhstan	Slovakia
Bosnia and Herzegovina	Kyrgyzstan	Slovenia
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Estonia	Montenegro	Turkmenistan
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Georgia	Norway	Uzbekistan
Germany	Poland	

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